National Cancer Control Programme (NCCP) 00207 · clinician draft

Weekly cetuximab

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First cetuximab dose

Active medicines: cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before cetuximab National Cancer Control Programme (NCCP) suggested premedication.
Dexamethasone	8 mg IV	60 minutes before cetuximab NCCP suggested mandatory infusion premedication; reconcile low-risk antiemetic support to avoid duplicate steroid.
Rescue antiemetic	As individually prescribed	If required Low emetic risk; use current NCCP/local guidance and account for the infusion steroid.

Additional phase-specific support

- Skin care and sun protection under local EGFR policy.
- First two doses require post-infusion observation; delayed-reaction education.
- Monthly electrolytes/Mg/Ca for two months after last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

NCCP 00207 v6 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Subsequent weekly cetuximab

Active medicines: cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before each infusion
Dexamethasone	8 mg IV	60 minutes before each infusion Reconcile antiemetic steroid; no automatic duplication.

Additional phase-specific support

- Premedication continues for subsequent infusions.
- Head/neck radiotherapy (RT) requires separate mucosal, skin, swallow and nutrition support.
- Post-treatment monthly electrolytes/Mg/Ca for two months.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

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Sources checked 2026-11-05 · SACTCheck v0.83.8 · Regimen-level drafts; prescriber and oncology-pharmacy verification required